

Examination of the Department of Veterans Affairs Systemic Failure of Its Electronic Health Record Modernization Program

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¹ *This White Paper is the result of a decades-long collaboration with hundreds of dedicated Department of Veterans Affairs employees, clinicians, informaticists, administrators, end-users, contractors, subject matter experts, and — most importantly — veterans. The current climate at the VA does not allow me to list them by name or title. I am solely responsible for the content of this document.*

This paper includes citations for supporting source material. The reader will find additional supporting documentation at <https://github.com/VETDOCS/Vetdocs>

Executive Summary

Introduction

Since 2018, The Department of Veterans Affairs (VA) has, been engaged in an Electronic Health Record Modernization (EHRM) program, that will, over the course of ten years, replace VA's current electronic health record, VistA (The Veterans Health Information Systems and Technology Architecture) with a commercial off the shelf (COTS) product whose lifetime budget was estimated at \$16 billion. The COTS product the VA chose was developed by Cerner, a company which is now owned by Oracle. The Cerner product was primarily developed to serve the goal of revenue maximization in for-profit private sector healthcare systems while VistA was developed by VA clinicians to meet the healthcare needs of VA's veterans, their caregivers, and staff.

VistA functions as the central nervous system of the nation's largest, fully integrated healthcare system administered by the Veterans Health Administration (VHA). VistA allows VHA to provide clinical care to 9 million veteran enrollees annually. These are some of the sickest and most complex patients in the United States. VistA also allows the VHA to

fulfill its Congressionally mandated teaching, research, and emergency preparedness missions. Among other things, VistA also provides critical data to the Veterans Benefits Administration (VBA).

All these services, and more, are now jeopardized by the rollout of the Cerner/Oracle EHRM which has been plagued with unresolved, and perhaps unresolvable, problems. The rollout in six small VHA facilities has caused significant harm to veterans, demoralized its clinical workforce, and consumed tens of billions of taxpayer dollars. Despite this, VA leaders are determined to deploy the EHRM in 13 new facilities this year, many of them large, complex medical centers.

This White Paper documents the origins of this EHRM program, the consequences of continuing it, and proposes a clear path forward that will assure that VHA and VBA staff have the information they need to deliver healthcare and benefits to our nation's veterans. ¹

Key Findings

- **Origins in DoD failure:** The origins of EHRM crisis can be directly traced to the Department of Defense’s troubled effort to replace its own Electronic Health Record system (AHLTA), which led DoD to acquire the commercial Cerner MHS GENESIS system — itself declared “neither operationally effective nor operationally suitable” during testing.²
- **Imposed on VA without justification:** In 2017, the White House Office of Government Innovation proposed that VA abandon its award-winning VistA system in favor of the same Cerner system used by the DoD. The decision was based on assertions that were demonstrably false: that VistA was a “legacy” system at the end-of-life, and that VA and DoD could only achieve interoperability if both agencies use a single vendor.³
- **\$16 billion sole-source contract with no requirements:** The Cerner/VA contract was awarded non-competitively in May 2018

²DoD Director of Operational Test and Evaluation reports on MHS GENESIS: 156 critical deficiency incident reports filed; system declared “neither operationally effective nor operationally suitable.” Three simulated cyberspace attacks successfully executed against the system with no failover capability identified. See GAO-24-106187 (Note 11) and GAO-21-571 (Note 10).

³ A legacy system is defined as one that has been in operation for a long period of time, serves its purpose, but the technology of current systems has advanced.

without defined requirements, Key Performance Indicators, a lifecycle budget, or an implementation schedule.⁴

- **Delays and Failures in implementation:** At five of the sites where the system was deployed, productivity fell by up to 50%, 83% of staff reported worsened morale, and 62.5% considered leaving. The system has been linked to 149 documented cases of veteran harm and, potentially, five veteran deaths.⁵⁶

Cost estimates did not reflect real costs which could exceed \$49.8 billion: An independent Institute for Defense Analysis (IDA) lifecycle analysis found the true cost of completing the EHRM program would be \$49.8 billion — more than three times the stated \$16 billion budget. Notably this figure excludes the 51% of VistA’s non-clinical functionality that the program does not address and which must be replaced.⁷ Current cost estimates also fail to reflect the hundreds of thousands of clinician hours devoted to implementation, conversion, or testing. Nor do they take into account the medical care delays or errors associated with the clinician

⁴VA EHRM Sole-Source Contract: Non-competitive award to Cerner Corporation, May 2018. Initial value \$10 billion, subsequently expanded to \$16 billion. VA established budget: \$10B EHRM costs + \$6B VistA sustainment during transition.

⁵VA Staff Survey, Mann-Grandstaff VA Medical Center Spokane, Washington; August–September 2021 (released November 2021); Survey of 833 clinicians and staff, conducted after 45,000 additional training hours and deployment of 400+ support personnel. Available at: <https://github.com/VETDOCS/Vetdocs>

⁶VA Office of Inspector General Report #22-01137-204, “The New Electronic Health Record’s Unknown Queue Caused Multiple Events of Patient Harm,” July 2022. Available at: <https://www.va.gov/oig/pubs/vaoig-22-01137-204.pdf>

⁷Institute for Defense Analysis (IDA), VA EHRM Lifecycle Cost Analysis, October 2022. Commissioned by VA at the request of Congress. Realistic lifecycle cost estimate: \$49.8 billion (vs. VA’s stated \$16 billion). Available at: <https://github.com/VETDOCS/Vetdocs>

time that are not devoted to care, but rather to implementation of the new EHR.⁸

VistA is not an archaic “legacy” system but is one that has been continuously updated and is in the process of being migrated to the latest technologies In fact, VistA continues to be fully operational at 160 of 170 VA medical centers, has been successfully migrated to the VA Enterprise Cloud on AWS, (at a cost of just \$10M/year to host). At each of the 10 EHRM sites there is a full read-only VistA system in operation that can be restored to full operation within days via a simple programming switch.⁹¹⁰ This trusted and reliable system is ready to resume full operations within weeks.

Recommendations

1. **Immediately halt** all further EHRM deployment and commission an independent operational review. This review must determine whether a COTS product can safely and effectively meet the totality of current and future VA requirements, at what cost, and in what

⁸ Koppel, Ross “Estimating the United States cost of health information technology.”

⁹VistA Fact Sheet 2026, Veterans Health Administration, March 11, 2026. Available at: <https://github.com/VETDOCS/Vetdocs>

¹⁰VA Enterprise Cloud (VAEC) migration completed FY24. All VistA systems hosted on Amazon Web Services (AWS). Annual total hosting cost: \$10M/year. Meets highest standards of VA, DoD, FISMA, NIST, and HIPAA security. See VistA Fact Sheet 2026 (Note 1).

timeframe. The review should compare these findings to the costs and timeframe needed to accomplish the same requirements by continuing to modernize and upgrade VistA.

While this review is taking place:

2. **Revert** all existing EHRM sites to VistA within 4–6 weeks using the VA’s own tested contingency plan.
3. **Restore** full funding and staffing to the VistA Program Office.
4. **Accelerate** the VA Health Cloud / VICS modernization plan as the strategic path forward for VistA.¹¹

¹¹VA Health Cloud: Centralized Veteran Integrated Care, Veteran Integrated Program V18-00154-000, U.S. Department of Veterans Affairs, Office of Information Technology. Available at: <https://github.com/VETDOCS/Vetdocs>

Background: Creating the Technological Foundation of the Veterans' Healthcare System

The Veterans Health Administration (VHA) began the process of inventing the modern Electronic Health Record (EHR) in 1978. This process resulted, in the early 1980's, in the deployment of the Decentralized Hospital Computer Program (DHCP) to 20 Veterans Medical Centers. DHCP was built on seminal work done a decade earlier at Massachusetts General Hospital using the Mumps (M) operating environment. DHCP — later renamed VistA — was and remains utterly unique because it was designed **by clinicians, for clinicians**, with programmers working in the service of clinical, patient care needs.¹² It allowed the clinical end user to define the business processes and the workflows, resulting in a clinician friendly, supportive user interface.

In 1996 the VA renamed DHCP. It was called the Veterans Health Information Systems and Technology Architecture (VistA). In 1997, The VA introduced the Computerized Patient Record System (CPRS). CPRS was significant because it integrated orders, notes, labs, imaging, and medications into a single unified view — making it one of the most advanced electronic health record systems in the country at the time.

¹²VistA History, VA Office of Information and Technology. Available at: <https://www.oit.va.gov/about/history.cfm>; Wikipedia: <https://en.wikipedia.org/wiki/VistA#History>; World VistA: https://www.worldvista.org/AboutVistA/VistA_History

It became widely admired in health IT circles and influenced the broader push toward EHR adoption in American healthcare that culminated in the HITECH Act of 2009. citation

In survey after survey, users have stated the system helps them to produce better health outcomes, create safer care environments, increase productivity as well as higher job satisfaction. Over the four decades since its introduction, VistA has earned the Smithsonian Award for best use of Information Technology in Medicine and has consistently received the highest physician satisfaction scores in national EHR surveys.¹³ In 2014 and again in 2016, national surveys of over 15,000 physician users of EHRs rated VistA with the **highest overall satisfaction rating in the United States**. Over 65% of all physicians trained in the U.S. rotate through the VHA and use VistA, making it the most widely known EHR in the country. Because Vista is already the most familiar EHR system to US clinicians, this obviates the need for extensive training in its use.

As a taxpayer-funded public domain software, VistA has been adopted as the foundation for public and commercial health systems worldwide. In 1985 both the U.S. Department of Defense (DoD) and Indian Health Service (IHS) adopted VistA as the basis of their national health information systems. Jordan, Finland, and several other nations use modified versions of VistA as their national health record system today.¹⁴

¹³Medscape EHR Report 2016: Physicians Rate Top EHRs — Survey of 15,285 physicians across 25 specialties. Available at: <https://www.medscape.com/features/slideshow/public/ehr2016>

Section I: What Is VistA?

Definition and Architecture

VistA is both a **technology** and an **architecture** that integrates all applications, all data, all workflows, and all interfaces within a single shared transactional database. VistA is built on the M (Mumps) technology — the same database-integrated language used by Epic, Meditech, Fidelity, TD Ameritrade, and most ATM networks in the United States and Europe.¹⁵

A precise working definition of VistA is that it is an integrated application database comprising exclusively M technology, providing a single integrated platform of **198 active M-based applications** integrated within a single shared M database. All VistA applications, business logic, workflows, and data are created, managed, stored, and processed within this integrated database. If an application is not built within the M language database, it is external to VistA — not a VistA application. The VA’s catalog of non-VistA applications is called the VA Technical Reference Model.¹⁶¹⁷

¹⁶VA VistA Documentation Library — documentation for nearly 200 integrated clinical, administrative, and infrastructure applications. Available at: <https://www.va.gov/vdl>

¹⁷VA Technical Reference Model — catalog of non-VistA applications. Available at: <https://www.oit.va.gov/Services/TRM>

Scale and Operational Reach

VistA's operational scale is remarkable:¹⁸

- **It has collected 400 million veteran-years of data** from 30 million patients.
- **It contains 198 integrated applications** — clinical (99), veteran-facing (19), administrative-financial (37), and infrastructure. (38)
- **It executes 200 million transactions per day** processed with millisecond latency¹⁹
- **It generates 50 million clinical reminders per day** and 4 million new orders, labs, images, and documents daily.
- **It is used by 371,000 healthcare professionals** at 1,298 hospitals and clinics.
- **It records 110 million veteran care encounters per year** across the United States.

Today, VA relies on VistA at **164 of its 170 medical centers**, supporting over 85% of the 9.6 million veterans enrolled in the VA health care system.

¹⁹ When referring to electronic health records latency is the delay between the time information is requested and when it can be viewed by the requester.

VistA Cloud Modernization

VistA is not, as claimed by those who seek to promote the Cerner/Oracle contract, a “legacy” system that is old, archaic and cannot be modernized. In fact, VistA has been actively modernized. In FY24, VA completed the migration of all VistA systems to the **VA Enterprise Cloud (VAEC)**, a federally certified commercial cloud operated by Amazon Web Services (AWS). VAEC meets the highest standards of security as required by VA, DoD, FISMA, NIST, and HIPAA.²⁰

The total annual cost of hosting all VistA systems in VAEC is **\$10 million per year**. The cloud platform makes available hundreds of new capabilities including AI, machine learning, and streaming analytics. The VistA Program Office has also employed AWS AppStream technology to transform CPRS into a **web-streaming service**, accessible on any browser or device anywhere. This is the same technology that delivers Netflix to hundreds of millions of viewers daily.²¹

The VA Health Cloud: VistA’s Strategic Future

The VistA Program Office has developed the **VA Health Cloud** (also called Veteran Integrated Care Services, or VICS): a comprehensive plan to incrementally centralize the functionality of all 130 VistA systems into a

²¹ AWS AppStream technology employed by the VistA Program Office to transform CPRS into a web-streaming service, enabling browser-based access across all devices and locations — comparable to how Netflix streams content. See VistA Fact Sheet 2026 (Note 1).

single, secure, federally certified commercial cloud service. This approach:²²

- Centralizes all 130 VISTA instances to a single integrated cloud service
- Preserves all VA- and veteran-specific services not available through generic commercial EHRs
- Provides backward-compatible continuity of care through RPC Emulation during transition
- Enables “Veteran Care Everywhere” — integrated care between any VA and community clinician, to any veteran, anywhere, through centralized cloud-based VICS
- Retires the 130 individual VISTA systems service-by-service with no disruption of care
- Eliminates residual and redundant legacy infrastructure and dramatically reduces maintenance costs

Section II: VistA's Proven Track Record

VistA's operational capabilities are documented in multiple independent assessments:

Clinical Performance

- VistA is available to clinicians over **99.999%** of the time, exclusive of scheduled maintenance windows
- Over **60% of clinical end-users** believe VistA enables them to deliver high-quality care — compared to fewer than 5% for the new EHRM system.²³

CPRS received the **highest physician rating (3.7/5.0)** of any EHR in the 2016 Medscape survey of 15,285 physicians across 25 specialties.²⁴ In 2018, an informal survey of VistA users favored VistA.²⁵

- Interoperability

²³KLAS Research / Arch Collaborative, VA EHR Modernization Survey, November 2022. Survey of 1,420 VA EHR end-users, benchmarked against 280 healthcare organizations worldwide. Available at: <https://github.com/VETDOCS/Vetdocs>

²⁵ **Jessica Davis.** “The results are in: Readers say VA officials should keep VistA EHR, not sign with Cerner.” Healthcare IT News. April 6, 2018.

<https://www.healthcareitnews.com/news/results-are-readers-say-va-officials-should-keep-vista-ehr-not-sign-cerner>

- VistA is connected to over **450 national and regional Health Information Exchanges (HIEs)**, making it the most connected EHR in the United States.²⁶
- Through ONC-certified Health Information Exchanges (HIEs), VistA is already interoperable with **thousands of third-party EHRs** including all Cerner commercial customers.
- VistA has allowed the VHA to have earned the distinction of being one of only **13 healthcare systems in the U.S.** to achieve HIMSS Health Level 7 certification — the highest level of health record integration.²⁷

Fiscal Efficiency

- Total cloud hosting cost: **\$10M/year** for all 130 VistA systems on AWS.²⁸
- VistA serves as the system of record for **74+ Veterans Benefits Administration programs** providing \$91 billion in benefits to 5.2 million veterans annually.

VistA handles **200 million transactions per day** with millisecond latency and gigabit-speed processing.

Section III: The Genesis of the Crisis — DoD's AHLTA Failure

²⁶ONC Health Information Exchange standards; VA connected to 450+ nationally certified HIEs enabling interoperability with thousands of third-party EHRs including Cerner. See ONC-certified Health IT Product List (CHPL).

²⁷HIMSS Electronic Medical Records Adoption Model (EMRAM) Stage 7 — highest level of healthcare IT integration. VistA is among only 13 healthcare systems in the U.S. to achieve HIMSS Health Level 7 certification.

Despite its success, VistA has been portrayed as an archaic, out of date, and failing Health Information System. How did this misleading and inaccurate depiction of VistA develop? To answer this question, we have cycle back to the year 2000, when the Department of Defense decided to revamp its own electronic healthcare record. This historical analysis allows us to clearly see how a perhaps well-intentioned effort to solve a serious problem in one part of the U.S. federal government has caused a catastrophic problem in another part of the federal government.

DoD's Failed AHLTA Experiment

In 2000, the Department of Defense appropriated \$6 billion to replace the Composite Health Care System (CHCS), which had been deployed across all military health facilities, with a system of their own design called the Armed Forces Health Longitudinal Technology Application (AHLTA). The replacement program was plagued with problems throughout its development and deployment. Nine years after it was launched, the title of a congressional hearing clearly described the problem and raised the obvious question: **"AHLTA IS 'INTOLERABLE' — WHERE DO WE GO FROM HERE?"**²⁹

To answer that question, the Assistant Secretary of Defense for Health Affairs, Dr. Ward Cassells, surveyed DoD clinicians and found widespread dissatisfaction with AHLTA. He created the "Way Ahead" office and

²⁹U.S. House Committee on Veterans' Affairs hearing: "AHLTA is 'Intolerable' — Where Do We Go from Here?" (~2009). Congressional Record.

appointed two DoD physicians to lead an 18-month Military Decision Making Process (MDMP) under FM 6-0 with 34 stakeholders from all DoD branches.³⁰

In 2011 the group issued its final “Way Ahead” Report. The report came to four conclusions:

5. The DoD had no ability or interest in fixing AHLTA.
6. The DoD had no interest in building a new system.
7. The DoD had no interest in using the VA’s VistA.
8. The DoD recommended buying a Commercial Off-The-Shelf (COTS) product.

Why the DoD Rejected VistA

The DoD’s rejection of VistA requires closer examination. The rationale DoD offered — which was later recycled to justify the VA’s own abandonment of VistA is highly suspect. In a May 2013 memorandum, Secretary of Defense Hagel cited two primary distinctions when he directed DoD to pursue a commercially available EHR:³¹ He made the distinction between a GOTS (Government Off-The-Shelf) product like VistA and a Commercial Off the Shelf (COTS) product. DoD acquisition policy at the time was structured to favor COTS products. This

³⁰DoD “Way Ahead” Final Report, 2011. Outcome of 18-month Military Decision Making Process (FM 6-0) with 34 stakeholders from all branches of DoD, directed by Assistant Secretary of Defense for Health Affairs Dr. Ward Cassells.

³¹Secretary of Defense Chuck Hagel Memorandum, May 2013, directing DoD to pursue a commercially available EHR. See also: GAO-14-302. Available at: <https://www.gao.gov/products/gao-14-302>

classification, while legally convenient, obscured the fact that VistA’s underlying M technology is widely deployed commercially and that third-party vendors (including DSS and Medsphere) had commercialized VistA successfully. Indeed, the most common EHR in the nation, Epic, uses M technology.

Hagel also asserted that the DoD could not use VistA because it lacked what is known as a Meaningful Use certification. In government contracting and procurement processes, a meaningful use certification is designed to prevent a vendor from promoting an untried and untested product. As of 2013, the VA’s own VistA had not yet formally achieved “Complete EHR” status on the ONC-certified Health IT Product List. Commercial EHRs (Epic, Cerner) had full certification. Third-party VistA-based products *had* achieved Meaningful Use certification. The only reason the VA’s government-maintained version had not received this certification was because the government had not submitted it for the same process.³² This would allow the DoD to claim that VistA was ineligible for DoD use. In short, DoD rejected VistA on procedural grounds, not on a thorough analysis of its clinical merit. This distinction matters enormously because the same discredited rationale was subsequently used to justify imposing the same COTS system on the VA.

Building on a Failed Foundation --Project Genesis

³²GAO-14-302, Electronic Health Records: VA and DOD Need to Support Ongoing and Future Efforts to Achieve Their Interoperability Goals. U.S. Government Accountability Office. Available at: <https://www.gao.gov/products/gao-14-302>

It took DoD Acquisition three years (2013–2015) to prepare a competitive Request for Proposal. The primary requirement in this proposal was the purchase of a COTS product. This product would have to be delivered and integrated into the DoD system. DoD chose not to integrate the new system itself or have the software vendor do it. So, its request for proposal included the requirement for what is known as a system integrator to serve as an intermediary between DoD and the COTS vendor. This requirement while relieving DoD from direct management of the program added an additional layer of management, increased communication complexity, and added significant additional cost to the new program.

Four bids were submitted:

- IBM, the system integrator, proposed Epic
- CSC, the system integrator, proposed AllScripts
- DSS proposed open-source VistA
- Leidos, the system integrator, proposed Cerner

DoD disqualified the VistA bid as non-compliant, citing the "Meaningful Use" certification issue. DoD may also have rejected VistA because of its well-known cultural aversion to accept that another federal agency, the VA, had developed a superior product.

In July 2015, a 10-year contract (5 + 5 option years) — formally designated **MHS GENESIS** — was awarded to the lowest bidder, Leidos, for \$4.3 billion.³³

MHS GENESIS: A Troubled System from the Start

Multiple Government Accountability Office (GAO) reports and DoD operational testing assessments documented serious problems with MHS GENESIS long before it was proposed to the VA.³⁴³⁵³⁶

- **Patient safety risks:** DoD’s Director of Operational Test and Evaluation found 156 “critical deficiency” incident reports, with users questioning the accuracy of information exchanged between systems. The significance of this fundamental failure for a system responsible for clinical decisions cannot be overstated.
- **Cybersecurity vulnerabilities:** DoD successfully executed three simulated cyberspace attacks against MHS GENESIS, exposing serious security gaps. The system was found to have no alternate server site — meaning a successful cyberattack could take the entire system offline with no fallback.

³³MHS GENESIS contract award to Leidos, July 2015; 10-year contract (5 + 5 option years) valued at \$4.3 billion. Congressional Research Service report: <https://sgp.fas.org/crs/natsec/R45987.pdf>

³⁴GAO-21-571, Electronic Health Records: DOD Has Made Progress, Challenges in Program Management and Oversight Remain. U.S. Government Accountability Office. Available at: <https://www.gao.gov/products/gao-21-571>

³⁵GAO-24-106187, Electronic Health Records: DOD Has Deployed New System, but Challenges Remain. U.S. Government Accountability Office. Available at: <https://www.gao.gov/products/gao-24-106187>

- **“Neither operationally effective nor operationally suitable”:** Early operational testing resulted in the system failing both core readiness metrics. Network latency (delays in responsiveness), delayed equipment upgrades, and failures to respond to problems created by the software contributed to a system front-line medical staff found unworkable.
- **Unprecedented low user satisfaction:** GAO’s 2024 report found only 20% of MHS GENESIS users found the system efficient, (the lowest usability rating ever recorded for any EHR system) only 21% reported fast response times, and just 29% felt it helped deliver high-quality care — all dramatically lower than the legacy system it replaced.
- **Unresolved module failures:** The Dentrrix dental module has been plagued with problems since 2018. By January 2024, DoD escalated it to “severe” status with no remediation plan, schedule, or timeline — six-plus years of a broken module with no resolution path.

Despite these documented failures, DoD’s response was to keep deploying the system to new sites, prioritizing schedule over safety and functionality — a pattern that would repeat at the VA.

Section IV: The Contagion — VA’s Imposed Adoption

The White House Proposal (June 2017)

In June of 2017, the Trump administration created the White House Office of Government Innovation (OGI) which was led by presidential appointee Jared Kushner, President Trump’s son-in-law, who has neither government, healthcare, nor IT experience. Kushner approached the VA with a proposal that it abandon its award winning, best of breed VistA electronic health record and, despite its troubled history, adopt the DoD Genesis system, which, as outlined above, had years of documented, serious problems. Kushner, relying on the advice of three wealthy Mar-A-Lago members and Trump allies, also without any military, healthcare, government, or IT experience, supported and influenced the President to proceed with this strategy.³⁷³⁸

What was the stated rationale for this proposal?

The newly created Office of Government Innovation, under totally unqualified leadership, grounded its arguments for replacing VistA on two assertions:

³⁷ Armsdorf, Isaac. The VA’s Shadow Rulers’ Signature Program Is “Trending Towards the Red.” ProPublica. November 1, 2018. <https://www.propublica.org/article/va-shadow-rulers-program-is-trending-towards-red>

³⁸ Allen, Arthur. “A 40-year ‘conspiracy’ at the VA.” Politico. March 19, 2017. <https://www.politico.com/agenda/story/2017/03/vista-computer-history-v-a-conspiracy-000367/>

9. VistA was an old, legacy system at the end of its life cycle that needed to be replaced.
10. There was an “interoperability problem” between VA and DoD, and the only solution was for both agencies to purchase and implement a single COTS product referencing a single database.

Both assertions are demonstrably false. Neither was supported by any research or analysis.

Debunking the False Assertions

Assertion 1: VistA Is a “Legacy” System

VistA is not a legacy system by any reasonable definition of the term. A legacy system refers to systems that run on outdated hardware, using obsolete software and archaic technologies that don’t allow for innovation and modernization. Vista is now native to the Amazon AWS Cloud environment taking advantage of the inherent state of the art security, redundancy, and reliability that it provides. VistA is written in ANSI standard M language used by well over half of all modern Electronic Health Record systems as well as major banking, reservation and logistics systems worldwide. VistA uses the most modern web streaming technology available to give its user interface, CPRS, the look, feel and capabilities of the most advanced UI available. In short VistA has been transformed from a Legacy system to a modern Leadership system that

still delivers the “Best Care Anywhere” via a modern evolving, extensible architecture that is suitable for AI innovation.

All major EHR systems deployed in the United States today were originally developed in the late 1970s or early 1980s—but most have not kept up with VistA in terms of adopting new technologies, adding capabilities or upgrading security. :³⁹ Consider for example the following facts:

- Epic was founded in 1979.
- Cerner was founded in 1979.
- Meditech was founded in 1976.

Epic and Meditech are built on the *same M technology* as VistA. Today, VistA’s M technology is used by over 50% of all commercially deployed EHRs in the U.S. While VistA’s underlying technology has evolved over time, its fundamental, fast, non-relational, hierarchical access to data remains optimal for the complex, large-scale environments of healthcare, banking, and finance. The singular, underappreciated distinction between VistA and Oracle Cerner is that VistA belongs to veterans, not a corporation.

Assertion 2: Interoperability with the DoD Requires a Single COTS Product

This assertion reflects a fundamental misunderstanding of how interoperability works. In the private sector there are over 1,000 unique Office of the National Coordinator's (ONC) Certified Health IT products that create and share health data according to ONC data standards. There are dozens of EHRs developed by multiple vendors. Mandating that all doctors in the U.S. use a single vendor's system is not how to achieve interoperability. That is why the U.S. Office of the National Coordinator (ONC) for Health Information Technology does not, in fact, mandate that all physicians in the United States select and use a single vendor's system. It mandates the use of ONC-approved Health Information Exchange (HIE) standards to achieve interoperability across diverse systems.⁴⁰

VistA is already *the most connected and interoperable EHR in the United States*, with trusted connectivity to over 450 national and regional HIEs. These HIEs, in turn, exchange veteran health data with thousands of commercial EHRs across the country — including Cerner. Through these ONC-certified HIEs, VistA is already interoperable with any Cerner EHR customer. Had DoD successfully implemented MHS GENESIS, VistA would have been interoperable with it *by definition* — because VistA is already interoperable with Cerner. It is a tragic waste of \$50 billions of taxpayer funds to rip-

and-replace VistA to achieve “interoperability” with DoD when VistA is already interoperable with Cerner.

The \$16 Billion Sole-Source Contract

Despite the absence of any credible evidence to support the rationale for replacement, in May 2018 the VA awarded the largest non-military, non-competitive, sole-source contract in VA history to the Cerner Corporation. The contract to acquire and install the same MHS GENESIS system being deployed at DoD was initially valued at **\$10 billion** and subsequently expanded to **\$16 billion**.⁴¹

The Cerner contract was also awarded without:

Defining VA-based requirements.

- Developing Key Performance Indicators (KPIs).
- Establishing metrics to measure success.
- Determining a vetted lifecycle budget.
- Creating a detailed implementation schedule.
- Assuring that VistA could maintain its ability to fulfill essential functions for, among others, the Veterans Benefits Administration and VA-wide management systems.

- Consideration of the fact that VA had already implemented a long-term aggressive modernization program for VistA and the most comprehensive interoperability platform approach in the industry.⁴²

To summarize an essential point: This contract only included a replacement for the clinical support and records management aspects of VistA. What was not considered is that VistA serves as the *central nervous system of the entire Department of Veterans Affairs* — with more than 50% of its functionality used for non-clinical purposes and outside the scope of the Cerner/Oracle contract.

Section V: A Cascade of Failures — The VA EHRM Implementation

Initial Deployment: Spokane (September 2020)

In September of 2020, the VA launched its new EHRM system at one site - - the Mann-Grandstaff VA Medical Center in Spokane, Washington. Problems emerged almost immediately. Serious issues with the type, timing, and level of training, combined with inadequate technical support, were compounded by latency and reliability failures. Established workflows were disrupted by ad hoc workarounds. Workloads increased while productivity fell by as much as **50%**. The new system crashed or

⁴²Open Health News, “VistA Evolution — What’s Wrong with This Picture?” Available at: <https://www.openhealthnews.com/story/2014-07-27/vista-evolution-whats-wrong-picture>; Note: VA’s “VistA Evolution” program (2013–2017) was unrelated to VistA itself — it replaced the Joint Longitudinal Viewer (JLV) with eHMP at a cost of \$350M.

froze sometimes several times a day, bringing modern healthcare to a halt. These problems have persisted for over five years.⁴³

Strategic Pause and Resumption (April 2021)

In April 2021, new VA leadership, under the Biden administration, called for a 12-week strategic pause to evaluate lessons learned. After the pause, and despite the documented problems at Spokane, VA elected to resume installations at four additional hospitals,

The Spokane Staff Survey (November 2021)

In November 2021, the VA released a survey of 833 clinicians and staff at the Mann-Grandstaff Spokane VAMC, conducted in August and September 2021. After a year of remediation efforts including 45,000 additional person-hours of training and the assignment of over 400 support personnel, the survey revealed devastating problems.⁴⁴

Morale:⁴⁵

- Improved — 0.7%
- Not changed — 16.0%

⁴³ Orion Donovan Smith at Spokane's Spokesman Review has done multiple articles on the problems of the Cerner HER rollout at this hospital. The most recent article summarizing its problems was written in collaboration with the Washington Post. Butler, Desmond and Donovan Smith, Orion. "VA staff flag dangerous errors ahead of next health records expansion." The Washington Post. December 3, 2025. <https://www.washingtonpost.com/investigations/2025/12/03/veterans-administration-va-hospitals-health/>

- **Worsened — 83.3%**

Job satisfaction:

- Improved — 0.7%
- Not changed — 20.8%
- **Worsened — 78.5%**

Considering leaving VA employment:

- No — 37.5%
- **Yes — 62.5%**

Level of burnout:

- Improved — 0.5%
- Not changed — 18.6%
- **Worsened — 80.9%**

Confidence using the EHRM system for job tasks:

- Strongly Disagree/Disagree — 61.7%
- Neutral — 22.4%
- **Agree/Strongly agree — 15.8%**

In every question and category, the results were consistent: near-universal dissatisfaction from the clinical staff who relied on this system to care for veterans.

VA Office of the Inspector General (OIG) Report: Unknown Queues and Patient Harm (July 2022)

In July 2022, the VA's Inspector General Office issued Report #22-01137-204: *"The New Electronic Health Record's Unknown Queue Caused Multiple Events of Patient Harm."* The report described 60 ongoing patient safety concerns across 9 healthcare delivery domains at the five EHRM sites.⁴⁶

The OIG found thousands of medical orders for laboratory services, pharmacy medications, specialty care referrals, outside referrals, consultations, follow-up care, rehabilitation services, and appointment scheduling that were routed, not to their intended recipients but to an unknown number of undetectable locations dubbed "**unknown queues.**" The system did not alert the sender or the intended recipient that these orders had not been delivered. At least **11,000 such orders** were discovered. While the system vendor was aware that these unknown queues existed, the VA was never notified.

The OIG found, and the VA acknowledged, at least **149 cases of documented harm** to veteran patients resulting from unknown queue issues. In September and October 2022, over **71,795 veterans** received letters from the VA advising them that they may have been harmed and should review their electronic health record for errors.

The VA is aware of **five veteran deaths** to which this system may have contributed to or caused the outcome. It cites ongoing investigations as the reason for not providing further detail.

A Veteran's Story: Charlie Bourg

One case has become emblematic of these failures. Charlie Bourg, a 71-year-old Army flight-qualified crew chief who served in Vietnam, was diagnosed with **stage 4 terminal prostate cancer** that has metastasized.

In December of 2020, Bourg's primary care clinician entered a referral into the EHRM system for a urology follow-up after an elevated PSA test. She told him a urologist would review his record and contact him if follow-up was required. For over a year, he heard nothing. During a routine office visit 13 months later, his physician discovered that her referral had disappeared into an "unknown queue. " The referral had never reached the urology department, and no one was notified. Charlie's elevated PSA number had, by then, led to terminal cancer. He is now living on borrowed time.

Deployment Suspension and the Inexplicable Decision to Continue

As a result of the OIG report, the VA suspended all further EHRM deployment in October 2022 pending resolution of documented deficiencies. However, the VA refused to suspend use of the system at the five sites where it was already deployed. This decision has never been adequately justified.

The VA’s only rationale for continuing to use a system it had deemed unfit for further deployment was that “adequate manual safeguards” had been implemented.

Specifically, VA leaders required that *individual clinicians manually verify that every order they place has been received by its intended recipient*.

This regression to the kind of past practices that were routine before the advent of electronic health records is almost unprecedented. It has imposed an additional burden on a workforce already experiencing years of attrition, transfer, retirement, and a hiring freeze due to local budget constraints.

The KLAS / Arch Collaborative Survey (November 2022)

In November 2022, the Arch Collaborative — a healthcare industry group formed by KLAS Research — released a comprehensive survey of the VA EHRM program benchmarked against 280 healthcare organizations worldwide. Its findings confirmed and amplified the internal survey data:⁴⁷

- The VA EHRM system was the ***lowest-performing EHR*** compared to all 280 healthcare organizations in the global benchmark
- In contrast, of 779 VA nurses using the existing VistA EHR, **64%** believe VistA enables them to deliver high-quality care

- Of 641 VA nurses using the new EHRM system, only **6%** believe the new system enables them to deliver high-quality care
- Across all 1,420 VA EHR end-users surveyed: **62.8%** of VistA users vs. **4.8%** of EHRM users believe their system enables delivery of high-quality care
- This approximately **13-to-1 ratio** favoring VistA was consistently maintained across every question and specialty surveyed

The True Cost: IDA Lifecycle Analysis (October 2022)

In October 2022, the Institute for Defense Analysis (IDA) — a private nonprofit managing three Federally Funded Research and Development Centers — released a lifecycle cost analysis commissioned by VA at Congress’s request. The VA had been operating under a stated lifecycle budget of \$16 billion (\$10B for EHRM costs, \$6B for VistA sustainment during transition). Congressional oversight committees had expressed skepticism about that figure.⁴⁸

IDA’s findings represented a clear confirmation of Congressional concerns. The analysis documented that a **realistic lifecycle budget for successful completion of the VA EHRM program is \$49.8 billion** — more than three times the stated budget. IDA also noted that this figure *excludes* many administrative and management capabilities currently

provided by VistA that are out of scope for the EHRM program, required rewrites and upgrades, and future VA infrastructure requirements.⁴⁹

Section VI: The Hidden Crisis — VistA’s Non-Clinical Functions

Perhaps the most consequential misconception surrounding the EHRM program is the assumption that VistA is simply a clinical records system. The EHRM program attempts to replicate roughly **49% of current VistA functionality** — the clinical portion. The other **51%: mission-critical applications** serving the entire department are now essentially orphaned.

Veterans Benefits Administration

VistA provides critical data to at least **74 Veterans Benefits Administration (VBA) programs** responsible for providing \$91 billion in life-sustaining benefits to over 5.2 million veterans annually. These programs include:

- Disability compensation and pension
- Education benefits (GI Bill)
- Home loan guarantees
- Vocational rehabilitation

⁴⁹Fierce Healthcare, “VA to spend \$4.9B maintaining EHR over next decade.” Available at: <https://www.fiercehealthcare.com/tech/va-to-spend-4-9b-maintaining-current-ehr-over-10-years-while-it-rolls-out-cerner-system>; Meritalk, “VistA Cost and Migration Concerns Emerge.” Available at: <https://www.meritalk.com/articles/as-v-a-retires-vista-cost-and-migration-concerns-emerge>

- Life insurance
- Survivor benefits

All these programs rely on access to accurate, up-to-date, and consistent data created and maintained in VistA's clinical components. When the clinical portion is migrated to a separate EHRM system, the data pathways that feed these VBA programs must be redesigned. This remains outside the current EHRM contract's scope and budget and will thus involve enormous cost over at least a decade.

VA-Wide Management and Financial Systems

VistA also serves as the authoritative system of record for:

- VA's budgeting, finance, auditing, fiscal controls, and reporting
- Logistics and supply chain systems
- Human resources and personnel management
- Planning and organizational management
- Research and institutional knowledge systems
- Hundreds of veteran-specific and VA-specific mandates established by Congress

Each of these interfaces must be redesigned, rebuilt, and tested to assure that they function effectively in the new EHRM environment. This work is costly, time-consuming, potentially disruptive, and entirely outside the scope and budget of the current contract. Even temporary workarounds to

keep these management systems functioning will require maintaining the current version of VistA in full production mode for **as long as 10 years** after any EHRM implementation.

Additional issues militate against continuing the EHRM program. These include the following:

- **The New EHRM program is functionally experimenting on veterans without their consent:** The VA has used EHRM sites to test system changes, fixes, and workarounds in a live environment on real patients — without first validating these changes in a full-scale test environment using real data and comprehensive use-case simulations. The VA has neither advised veterans of this practice nor obtained their consent to serve as test subjects.
- **The New EHRM program included no scalability validation:** The VA has never modeled or simulated whether the underlying EHRM technology can scale to meet the demands of the largest integrated healthcare system in the United States. The system has experienced latency, routing errors, and reliability failures at the relatively modest scale of its initial rollout facilities. Its behavior at full VA scale has never been tested.
- **The program poses a highly problematic vendor lock-in issue:** Currently the VA controls its own technological destiny by virtue of full ownership of VistA’s intellectual property, source code, data, and

architecture. It can decide what and when changes to the system are needed, set priorities and respond to changes as required. By adopting the EHRM program, the VA surrenders all these controls and becomes just another customer. Once the EHRM is fully implemented, the VA will have little leverage with the vendor, no reasonable alternatives, and must accept whatever licensing and cost terms that are imposed.

- **Clinical purpose vs. commercial purpose:** VistA was created by clinicians for clinicians, with decades of innovation toward the goal of improving delivery of medical care. The underlying EHRM COTS product is principally designed to *optimize billable event capture* in the commercial for-profit hospital marketplace. This commercial focus has come at the documented expense of usability, clinical workflow support, and ease of use.
- **Vendor stability:** Recent statements about the possible sale of the EHRM vendor company raise legitimate concerns about the vendor's long-term commitment to the success of the VA's program. ⁵⁰
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- **The VA EHRM System presents significant security concerns.** The security and privacy implications of veteran data held in a proprietary commercial system warrant serious scrutiny. The EHRM system that the VA uses is controlled by DoD and exists in a legacy data center in

⁵⁰Swain, Gyana. Oracle may slash up to 30,000 jobs to fund AI data-center expansion as US banks retreat. Jan 30, 2026. CIO. <https://www.cio.com/article/4125103/oracle-may-slash-up-to-30000-jobs-to-fund-ai-data-center-expansion-as-us-banks-retreat.html>

Kansas. That data center was abandoned by Oracle after Oracle bought Cerner. The Cerner HQ and data center is now an empty ghost town. All other Cerner customers moved to Oracle's data center in Virginia. DoD has refused to move out of this environment. A recent GAO-26-107492 report, issued October 2025, highlights serious security risks to the Department of Defense's Military Health System (MHS).

⁵¹(Citation.)

Key Findings:

- GAO found that massive amounts of traceable data from personal and government devices, online communications, and defense platforms (ships, aircraft, etc.) can be aggregated into detailed digital profiles. These profiles can threaten personnel, their families, operations, and national security
- Policy gaps: Two of five OSD offices have not issued policies on public accessibility risks, and the policies issued by the other three are too narrowly focused, exclude many stakeholders, and do not cover all relevant security areas
- Training deficiencies: Nine of ten DOD components do not consistently train personnel on the risks of public digital information

⁵¹ GAO. Information Environment: DOD Needs to Address Security Risks of Publicly Accessible Information. **GAO-26-107492. October. 7, 2025.** <https://www.gao.gov/products/gao-26-107492>.

- Assessment gaps: Eight of ten components fail to conduct comprehensive threat assessments in force protection, insider threat, mission assurance, and operations security

Publicly accessible digital information can be exploited by malicious actors to:

- Identify and target personnel
- Disrupt operations
- Compromise mission assurance
- Undermine national security

That this situation remains largely unrecognized almost 10 years after the initial decision to pursue a rip-and-replace strategy — and that no known plan or program exists to address it — is a cause for serious concern about the long-term viability of the EHRM program.

Section VII: Despite All Its Problems, Why Has the EHRM Program Been Continued?

There are several theories as to why the VA has continued to use a system that causes harm to veterans, demoralizes its clinical staff, reduces productivity, increases costs, and has failed every test of a modern, mission-critical, life-dependent system:

- **Contractual entanglement:** Severe contractual issues involving both DoD and VA would carry large financial and operational implications for both departments if the program were terminated. This assumption is largely erroneous because the DoD Cerner contract ended and the VA's contract is negotiated annually.
- **Sunk cost fallacy:** Decisions are influenced by the enormous investment of cost, time, and professional reputations already expended, and the hope that continued investment will eventually yield success. Without any concrete evidence that success will be the eventual result, continuing with the Cerner/Oracle contract commits the U.S. taxpayer to pouring good money after bad.
- **False narratives and political involvement:** The rationale for the EHRM program was built on assertions that have never been validated. Individuals who championed those assertions may resist acknowledging this critical fact.

- **Leadership inability to admit error:** Acknowledging the scale of this failure requires a degree of institutional courage that has not yet been demonstrated by those in a position to act.
- **The myth of “no fallback”:** Perhaps the most pernicious theory is that there is no fallback position available to the VA. *This is patently false.* As documented in the next section, a safe, tested, and inexpensive reversion path exists and has been prepared.

Section IX: The Path Forward — VistA Modernization and the VA Health Cloud

What the VistA Program Office Has Already Achieved

While the VA’s attention and budget have been focused on the failing EHRM program, the VistA Program Office has pursued a long-planned, deliberately executed modernization program. Though woefully understaffed and chronically underfunded, this team has achieved remarkable results:

- Migrated all VistA clinical applications from legacy on-premises data centers to the *VA Enterprise Cloud (VAEC) on AWS* — with full functionality maintained and reliability, security, and access all improved.⁵²
- Implemented *AWS zero-trust security* and AWS Edge backup/redundancy across all VistA systems.
- Deployed *AWS AppStream technology* to transform CPRS into a web streaming service, enabling browser-based access on any device from any location — dramatically increasing CPRS usability.⁵³

- Maintained VistA's connection to over *450 Health Information Exchanges* — sustaining and advancing the most interoperable EHR in the nation.
- Developed a detailed plan for future improvements, code modernization, and transformations of VistA's integrated applications.
- Recognized the inherent suitability of the Mumps hierarchical data structure for *AI and machine learning* implementation — a capability fundamentally constrained by the relational data model of the EHRM system.

The VA Health Cloud: The Strategic Future

The VistA Program Office has developed the *VA Health Cloud* (Veteran Integrated Care Services — VICS) as the definitive modernization path forward. This is not a rip-and-replace strategy — it is an *incremental, service-by-service centralization* of VistA functionality to a single, secure, federally certified commercial cloud. VA Program V18-00154-000 achieved Initial Operating Capability in Q4 2018.⁵⁴

The approach uses an *RPC Router and Emulation layer* that provides backward-compatible continuity of care throughout the migration: CPRS continues to function unchanged while VistA functionality is incrementally centralized. As each service is centralized, the

corresponding functionality across all 130 VISTA systems can be safely decommissioned enterprise-wide, with no loss of care continuity and no disruption to clinical workflows.

The end state is a single, centralized, commercially maintained cloud service delivering integrated care between any VA clinician and any community clinician to any veteran, anywhere — *"Veteran Care Everywhere."*

The Reversion Plan: Ready Now

The VA sites currently using the EHRM system could be returned to VistA safely, at very modest cost, within *4 to 6 weeks*. All EHRM sites already operate a full production VistA/CPRS system in parallel. This is an operational necessity. Because such a small fraction of veterans' historical health records was transferred into the EHRM system, clinicians must access CPRS to obtain an accurate summary of lifetime care history. The EHRM system contains less than 10% of a veteran's record, and only for the most recent seven years.

The VA has prepared carefully for this eventuality. VA has created an *"active/inactive" switch* in VistA that transitions VistA from "read-only / hot standby" to "read-write / fully active" through a set of predetermined, pre-tested configuration changes — implementable within days. Reverting to VistA would rapidly restore healthcare integrity, reduce clinician stress, and dramatically reduce wasteful spending.

No extended retraining would be required. All VA clinical staff already know and use CPRS. Short refresher training would be all that is needed.

Conclusions and Recommendations

The Department of Veterans Affairs has had a struggling, out-of-date, underfunded, commercially oriented, generic EHR replacement program procured by another agency imposed upon it. The proposed replacement degrades the VA's ability to provide safe, accessible care to veterans and represents an existential threat to the viability of the VA. The documented consequences of this system are clear:

- **50% reduction in clinical productivity**
- **83% of clinical staff** reporting worsened morale
- **62.5% of clinical staff** considering leaving the VA
- **149 documented instances** of real harm to veterans
- **71,795 veterans** sent potential-harm notification letters
- **Potentially 5 veteran deaths** linked to system failures
- **\$49.8 billion realistic lifecycle cost** (vs. stated \$16 billion)
- **51% of mission-critical VistA** functionality orphaned with no remediation plan
- **Only 4.8% of EHRM users** believe it enables delivery of high-quality care (vs. 62.8% for VistA)

There is no ethical or practical rationale for continuing to experiment on veterans' lives, subjecting dedicated clinical staff to the stress and burden of a failed system, or spending unbounded amounts of taxpayer dollars on a program that has failed every test of a modern, mission-critical, life-dependent system.

VA leadership must summon the courage to acknowledge past errors and — despite any contractual, political, or personal consequences — act decisively to:

1. **Immediately halt all further EHRM deployment** and initiate an independent operational and safety review encompassing all active EHRM sites. This review must determine whether a COTS product can safely and effectively meet the totality of current and future VA requirements, at what cost and in what timeframe. The review should compare these findings to the costs and timeframe needed to accomplish the same requirements by continuing to modernize and upgrade VistA.
2. **Revert all EHRM sites to VistA** within 4–6 weeks using the VA's existing tested contingency plan. Brief refresher training is all that is required of staff who already know and use CPRS.
3. **Restore full funding and staffing to the VistA Program Office** to enable acceleration of the VA Health Cloud / VICS modernization

initiative — the proven, incremental, non-disruptive path to a fully modernized national veteran health information system.

4. **Address the 51% problem:** commission an immediate, comprehensive plan to ensure continuity of VistA’s non-clinical functions supporting the Veterans Benefits Administration and VA-wide management systems.
5. **Initiate a formal accountability review** of the decision-making chain that produced this outcome — from the initial false assertions to the sole-source contract to the continued operation of a system acknowledged to cause patient harm.

The veterans who served this nation deserve a healthcare system that serves them reliably, safely, and with the respect their sacrifice demands. The nation also depends on the VHA’s research, teaching, and emergency preparedness missions.

Sources and Reference Documents

All referenced documents are publicly available at <https://github.com/VETDOCS/Vetdocs>. The shortcut links below provide direct access to key sources:

- [VistA Fact Sheet 2026 \(VHA, March 11, 2026\)](#)
- [VA Health Cloud — Centralized Veteran Integrated Care \(Program V18-00154-000\)](#)
- [VA OIG Report #22-01137-204 — Unknown Queue Patient Harm \(July 2022\)](#)
- [GAO-19-125 — Electronic Health Records: VA Needs to Identify and Report System Costs](#)
- [GAO-14-302 — VA and DOD EHR Interoperability](#)
- [GAO-21-571 — DOD EHR Program Progress and Challenges](#)
- [GAO-24-106187 — DOD Has Deployed New EHR System, Challenges Remain](#)
- [CRS Report — MHS GENESIS Background and Issues for Congress](#)
- [VA VistA Documentation Library](#)
- [VA Technical Reference Model](#)
- [VistA History — VA Office of Information and Technology](#)
- [VistA History — Wikipedia](#)
- [Medscape EHR Report 2016: Physicians Rate Top EHRs](#)
- [Author Biography — Edward Francis Meagher](#)

Author Biography

Edward Francis Meagher

Edward Francis Meagher currently serves as the Chief Executive Officer of Vetegic, a corporate strategy and information technology consultancy specializing in the Federal healthcare informatics market. Immediately prior to this he retired as the Chief Operating Officer at Information Innovators Inc., upon the successful sale of the company. Previously he served as Vice President Healthcare Group at SRA International responsible for the strategic implementation of healthcare programs for the Department of Veterans Affairs and the Department of Defense Military Health System. Prior to that he served as Vice president for Healthcare Strategy for CSC where he was responsible for providing leadership, innovation, and strategic vision within CSC's North American Public Sector. In 2000, Mr. Meagher was nominated by President Clinton to serve as Assistant Secretary for Information and Technology for the Department of Veterans Affairs. Prior to this appointment, Mr. Meagher served as the

Special Assistant to the Secretary on matters relating to information technology. In August 2001 he was appointed as Deputy Assistant Secretary for Information Technology in the Department of Veterans Affairs (VA). As the senior career IT official in the Department, he also served as VA's Deputy Chief Information Officer, providing direction, oversight, and management of information technology operations of the federal government's largest civilian department, with a multi-billion-dollar budget and over 225,000 employees in VA medical centers, clinics, benefits offices and national cemeteries throughout the country. In June 2003 Mr. Meagher was appointed by the Secretary of Veterans Affairs to serve in the position of Acting Assistant Secretary for Information and Technology. In this role, he also served as VA's Acting Chief Information Officer, advising the Secretary of Veterans Affairs on all matters pertaining to information and technology management in the Department. In December of 2005 he also assumed the roles of Chief Technology Officer and Chief Information Security Officer for the Department of Veterans Affairs. His extensive career in both the public and private sectors in information technology and healthcare informatics spans over 40 years. Mr. Meagher has received numerous awards in recognition of his performance, leadership, and contributions in information technology, both in the federal arena and the private sector. Most notably, he was honored with a Federal 100 Award in 2001 and again in 2005. This award is presented to the 100 leaders who made a difference in federal information technology during the previous year.

He also received the coveted FCW Eagle Award in 2005 presented annually to a government executive who made a particular impact in the government IT community. Mr.

Meagher received the Distinguished Career Award upon his retirement from government. Mr. Meagher has served on numerous boards and committees both within government and in the private sector. Mr. Meagher was the president of AFFIRM and served on the board of the American Council for Technology. Mr. Meagher is a member of the Partnership for Public Service CIO SAGE community, and previously served on the executive committee of the Industry Advisory Council and served as the Chair of the ACT/IAC Vista Modernization Committee. Mr. Meagher is active in the veteran community and currently serves on the boards of the Aleethia Foundation, Service Member Support Foundation, Wounded Warrior Project Advisory Board, VETSports, Help Heal Veterans Foundation and the Blue Star Families. Mr. Meagher has for over 14 years organized and executed Operation Covert Santa in support of hospitalized service members and veterans at Walter Reed National Military Medical Center at Bethesda over the Christmas holidays and Operation Jumpstart in support of military to civilian transitioning servicemembers.

A Service-Disabled Air Force Vietnam Veteran, he was born in Brooklyn, New York and attended the University of Dayton, Dayton, Ohio. He is married to his college sweetheart, and they have 3 grown children and 6 grandchildren and lives in Great Falls, Virginia.

